

2. Menstrual Stigma and Harmful Cultural Beliefs

Deep-seated myths and taboos continue to surround menstruation in Ghana. In the Volta Region, qualitative evidence reveals:

- **Ritual Fears:** Persistent beliefs that menstrual blood can be used for spiritual or ritual purposes lead many girls to fear disposing of used menstrual materials.
- **Concepts of Uncleanliness:** Menstruation is often framed as "unclean," resulting in restrictive social, religious, and household rules.
- **Emotional Toll:** These beliefs foster a culture of secrecy and shame, which erodes self-confidence and increases anxiety.

3. Inadequate Menstrual Health Education

A lack of comprehensive knowledge among the wider community reinforces misinformation. Research involving adolescent boys shows that negative sociocultural beliefs often outweigh positive ones, leading to poor attitudes toward their female peers. While better education has been shown to improve attitudes, current levels of understanding among parents, teachers, and boys remain insufficient to break the cycle of stigma.

4. Cost and Access Barriers

Economic factors remain a primary barrier. Many girls cannot afford commercially produced menstrual products and therefore resort to less hygienic alternatives.. This not only causes physical discomfort but significantly increases stress and contributes to school absenteeism.

Policy Actions

To address the intersection of menstrual health and mental well-being, policymakers must implement a multi-sectoral approach involving the Ministry of Education, the Ministry of Health, and local stakeholders.

1. Integrate Menstrual Health into Mental Health Programs

Menstrual health should be officially recognized as a contributor to adolescent mental well-being. School health and counselling programs should include specific indicators to monitor and support girls’ menstrual health as part of their broader psychological care.

2. Prioritize Investment in School WASH Infrastructure

Schools must be equipped to provide a dignified experience. Investments should focus on:

- Clean, lockable toilets.

- Reliable water sources within or near changing areas.
- Safe and private menstrual waste disposal facilities.

3. Expand Comprehensive Menstrual Health Education

Education initiatives must move beyond girls to include boys, teachers, and parents. Curriculum and community outreach should explicitly address and debunk myths, taboos, and ritual-based fears to reduce the stigma that drives psychological distress.

4. Improve Access to Affordable Products

The government and its partners should explore several avenues to reduce the financial burden on families:

- Promote approved, high-quality reusable menstrual pads as a safe, affordable, and environmentally sustainable option.
- Subsidizing sanitary products.
- Implementing school-based menstrual product support schemes.
- Supporting local production initiatives to lower costs and increase availability.

5. Promote Community Dialogue

Engage traditional and faith leaders, parents, and youth groups to challenge harmful norms and promote supportive community attitudes toward menstrual health.

Policy Takeaway

Investing in menstrual health is a direct investment in mental health, education, gender equality, and adolescent development. When girls manage their periods with dignity, the risks of depression and anxiety decreases, allowing them to remain in school and reach their full potential.

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POLICY BRIEF
Menstrual Health Is Mental Health: Evidence and Policy Actions to Support Adolescent Girls in Ghana

Introduction

This policy brief presents evidence from the project “Exploring the Relationship Between Menstrual Hygiene Management and Depression Among Adolescents in Ghana,” conducted in five districts of the Volta Region. This brief summarizes the key findings from the research, outlines the factors driving the menstrual health–mental health connection, and presents practical policy recommendations for key stakeholders.

Executive Summary

Menstruation is a natural biological process, yet for many adolescent girls in Ghana, it remains a source of significant psychological burden. Girls who are unable to manage their menstruation safely, privately, and with dignity are significantly more likely to experience various forms of psychological distress, including depression, anxiety, stress, shame, and social isolation. Evidence indicates that menstrual health should no longer be categorized strictly as a sanitation or hygiene issue. Instead, it must be recognized as a fundamental component of mental health and adolescent development. Creating supportive menstrual health environments is essential for improving both educational and mental health outcomes for adolescent girls across the country.

Key Message

Unmet menstrual health needs, encompassing inadequate materials, lack of privacy, and poor disposal systems, are directly associated with poor mental health outcomes among school-aged girls.

The Scale of the Problem

Research conducted among 1,161 senior high school girls in five districts in the Volta Region of Ghana has revealed a staggering correlation between menstrual management challenges and mental health disorders. The study found substantial levels of distress among the student population, directly linked to their experiences with menstruation.

Statistical Overview of Psychological Distress

The following data represents the prevalence of mental health symptoms reported by adolescent girls in the Volta Region:

Condition	Severity Level	Prevalence
Depression	Mild symptoms	41.0%
Depression	Moderate symptoms	22.2%
Anxiety	Severe anxiety	22.8%
Stress	Above normal levels	38.3%

The Menstrual Health–Mental Health Pathway

The transition from biological menstruation to clinical psychological distress is driven by a specific pathway of unmet needs:

- **Inadequate Facilities & Products:** Lack of access to pads and clean toilets.
- **Stress and Fear:** Constant worry regarding potential leakage.

- **Social Consequences:** Shame, embarrassment, and fear of stigma.
- **Behavioral Impact:** School disengagement, absenteeism, and social isolation.
- **Clinical Outcomes:** Development of depression, anxiety, and chronic stress.

Evidence confirms that girls who face challenges relating to menstrual materials, privacy, and disposal are more likely to report these negative mental health outcomes.

Key Drivers of the Menstrual Health Crisis

1. Inadequate School WASH Facilities

Water, Sanitation, and Hygiene (WASH) infrastructure in schools is often insufficient. High school girls report low satisfaction with privacy and disposal arrangements. Common issues include:

- Poorly maintained and unhygienic toilets.
- Lack of reliable water supplies for personal cleaning.
- Absence of secure, discrete disposal systems for used menstrual materials.

Beyond Hygiene: The Menstrual Health-Mental Health Connection

THE CRISIS: A HIDDEN MENTAL HEALTH BURDEN



KEY FINDINGS FROM THE STUDY OF 1,161 SHS GIRLS

Key findings from a study of 1,161 SHS girls reveal that menstrual management challenges significantly affect the mental health of girls in Ghana's Volta Region.



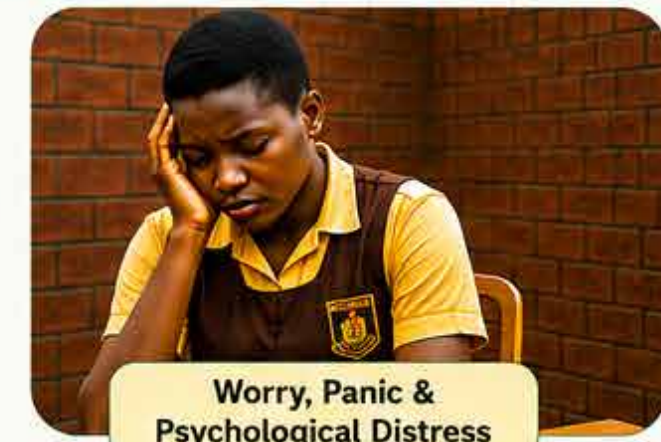
THE PATHWAY TO ISOLATION



Inadequate Facilities & Fear of Leakage



Shame & Social Withdrawal



Worry, Panic & Psychological Distress

THE SOLUTION: POLICY ACTIONS FOR CHANGE



INTEGRATE HEALTH PROGRAMS:

Merge menstrual health indicators into existing school mental health and counseling frameworks.



UPGRADE SCHOOL INFRASTRUCTURE:

Invest in clean water, private changing areas, and safe, well-lit toilets with menstrual waste disposal systems.



PROMOTE APPROVED, HIGH-QUALITY REUSABLE MENSTRUAL PADS:

Ensure access to safe, affordable, and environmentally sustainable reusable pads through schools and community programs. Train girls on proper use and care.



COMMUNITY-WIDE EDUCATION:

Target girls, boys, parents, teachers, and community leaders to dismantle myths and promote supportive attitudes and practices.



STRENGTHEN MENTAL HEALTH SUPPORT:

Train school counselors and teachers to provide confidential, girl-centered mental health support.